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Reply to Wideman et al.

Letter to the Editor:

We appreciate the authors' engagement with our work.² A primary objective of our study was to stimulate a critical discourse on the concept of pain-related suffering. In this regard, we value the opportunity for dialogue, although we were surprised by the letter's polarizing tone. We must explicitly address 4 critical points, as the arguments presented seem to foster misunderstandings that jeopardize a constructive and critical discussion.

1. Confusion of objectives: finding consensus in groups vs synthesizing the literature

Wideman et al. criticize our use of the term “consensus” and highlight methods such as the Delphi technique. We used the term to refer to “the state of the art,” or else “the conceptualization that most experts have arrived at.” The agreement that is achieved in Delphi and similar processes is another denotation of

“consensus” and none the less important. This distinction is crucial, as we do not claim that our research encompasses both kinds of consensus.

However, the critique by the authors reveals a broader misunderstanding of our chosen approach. The Delphi technique is indeed effective for reaching consensus within groups, potentially creating new concepts and definitions reflecting the opinion of the participating groups. Our aim, explicitly stated, was not to develop a new definition but to synthesize the existing literature into an inclusive definition (“*Instead of adding yet another theory of suffering, we offer a definition that brings together insights from different disciplines and can serve as a consensus definition against which new theories and operationalizations can be evaluated*” [p. 11]). Consequently, this definition represents a consensus in the sense of a synthesis of existing research, rather than the personal opinion of the authors.

This synthesis can then serve as input for other consensus methods, such as those mentioned by Wideman et al. We would welcome and feel honored if professional societies used our work as a basis for further developing a suitable definition, for instance, through Delphi procedures.

2. The false dichotomy: artificial intelligence vs patient involvement

Wideman et al. reject the use of artificial intelligence (AI)-based algorithms in analyzing the concept of suffering. They argue that (1) definitions require “*careful thought and consideration*,” and (2) must be developed in partnership with people living with pain. We appreciate this perspective, but we believe that this viewpoint does not contradict our approach or the use of AI in general.

First, all scientific work necessitates careful thought and consideration, and the use of AI is no exception. This is precisely why we chose a combination of manual qualitative analysis and natural language processing to examine the concept of pain-related suffering in the current literature, cross-validating our findings. Reducing our study to the mere use of AI would be a profound misinterpretation of our approach. Furthermore, we did not simply query ChatGPT about suffering based on its broad training data. Instead, we provided specific algorithms with concrete, comprehensible text input in a systematic procedure.

Regarding the second point, we must issue a cautionary note. The involvement of people living with pain in scientific projects is crucial for clinically meaningful progress. We draw attention to promising ongoing projects that address this issue (eg, <https://painstory.science>; see also <https://www.youtube.com/watch?v=ZHJyL5Gmw0I>). However, this does not devalue research without the direct involvement of people with lived experiences. Such polarization and the creation of a false dichotomy would be a fundamental error, akin to dismissing purely biological research as “inadequate” in the context of biopsychosocial issues.

3. Confusion over the specificity of the definition: pain-related and other kinds of suffering

Wideman et al. criticize that our definition does not “make any link to pain,” which they claim, “*undermines its value in differentiating pain-related suffering from other forms of suffering (eg, grief)*.” Wideman et al. argue that this lack of specificity in our proposed concept of pain-related suffering compared with other forms of suffering indicates the inadequacy of our approach. To repeat, our goal was to conceptualize suffering in the context of pain by analyzing the usage of this term in the pain literature. It is specific insofar as it is based on this specific literature. This approach

Table 1
Pain-related suffering.

Definition
A severely negative, complex, and dynamic experience in response to a perceived threat to an individual's integrity as a self and/or identity as a person.
Expansions
Pain-related suffering is experienced to different degrees on a physical, spiritual, existential, personal, social, cultural, affective, and cognitive level.
Pain-related suffering is often associated with feelings of loss, lack of control, illness, alienation, and reduced quality of life. However, none of these can in itself be considered suffering, if it is not experienced as a threat as defined above.
Pain-related suffering is often a long-term experience but not necessarily so.
Pain and pain-related suffering are related but distinct phenomena. Either can cause or enhance the other.
The experience of pain-related suffering depends on the complexity of the affected individual. Newborns do suffer, but their suffering is not taking place on, eg, an existential or spiritual level, and they are not threatened as persons but rather as selves.

The wording in this table has been slightly modified from the original version; "and/or" has been used instead of the original "and" to align with the original meaning and to avoid misunderstanding.

neither requires nor precludes that our definition could be applied to other forms of suffering. On the contrary, as we discuss in our article (p. 12), our approach can be applied to different text corpora (for instance the literature on suffering from dementia). On this basis, differences between conceptualizations of suffering within different research areas could be discussed and the specificity of pain-related suffering would become more evident. Therefore, we acknowledge the need for more specificity but argue that this does not undermine at all the value of our definition, which is a first step to more specific definitions of different kinds of suffering.

Unfortunately, the authors remain vague about how conceptualizations of suffering should discuss the "link to pain" according to their criticism. This is even more puzzling, as our definition explicitly mentions the relationship between suffering and pain (Table 1). If the authors intend to say that within suffering there must be an intentional relatedness to or representation of pain (and not for instance dementia or grief), we would like to point out that this kind of analysis is beyond the scope of this study. However, we are pleased to refer to recent philosophical publications in which the experiential relationship of pain and suffering is examined in detail.¹

4. Misunderstandings regarding our conceptual framework

Wideman et al. criticize our framework as (1) "a preliminary 'laundry list' [to] inform factor analysis" that (2) lacks theoretical underpinning. First, we fail to understand how the potential to inform factor analysis is an argument against our conceptual framework. Second, we would like to point out once again that we aimed to bring together conceptualizations from the literature and systematically summarize them into the dimensions of our framework so that indeed each dimension (and its descriptors) is backed up by specific articles from the literature. We are happy to share details about this upon request. There is no doubt that a discussion about the theoretical foundation of each dimension is vital to further research based on our framework.

Although surprised by the tone of their criticism, we would like to express our sincere gratitude to Wideman et al. for highlighting aspects of our approach that could be misinterpreted. We hope that the explanations provided here will offer greater clarity and foster further constructive discussion.

Conflict of interest statement

The authors have no conflicts of interest to declare.

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The revised pediatric chronic pain grading: built on outdated foundations, overlooking newer pediatric versions

Letter to the Editor:

We have read with interest the article by Grothus et al.³ recently accepted for publication in *PAIN*. Although the topic of the paper is important, we have identified a number of concerns and issues with this paper that we would like to share.

The paper presents findings from a study that sought to evaluate the validity of a revised pediatric version of the Chronic Pain Grading Questionnaire (CPGQ). This revised version of the CPGQ, as described by the authors, "...is a further development of the adolescent CPGQ by Wager et al.,¹⁰ which is based on the adult CPGQ by von Korff et al.⁸" The adult version of the CPGQ was developed in 1992 to aid in the classification of adults with chronic pain, based on their ratings of pain intensity, pain-related disability, and disability days. In addition to pain intensity, pain-related disability, and disability days, the revised version developed by Grothus et al.³ classifies chronic pain also based on pain-unrelated emotional impairment. The assessment of this domain was not in the original version⁸ or subsequent pediatric versions of the questionnaire.⁴ Furthermore, one of the arguments the authors use to develop the revised version of the CPGQ is that the current most updated version of the CPGQ